4.2 Hypertensive Crisis*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I have a severe headache and blurred vision." **Objective** • BP 210/130 mmHg • Nausea • Confused	**ACTUAL** Acute pain related to severely elevated blood pressure as evidenced by severe headache, blurred vision and nausea.	• BP lowered gradually as prescribed (usually no more than 25% in the first hour) with no neurological decline.	• Nurse her in a quiet room with the head of the bed raised. • Measure BP every 5–15 minutes during IV therapy. • Run the IV antihypertensive by pump as prescribed. • Assess neurological status hourly. • Report chest pain, worsening confusion or seizures.	• Goal reached when BP falls as planned and neurological status holds steady.
Subjective • "I am frightened." **Objective** • Restless • HR 112/min	**ACTUAL** Excessive anxiety related to sudden severe symptoms as evidenced by fear and restlessness.	• Feels calmer and rests quietly within 1 hour.	• Explain procedures and monitoring in plain words. • Stay with her through the acute phase. • Keep visitors, noise and light to a minimum. • Coach slow breathing.	• Met when she is calmer.
Subjective *No specific complaint* **Objective** • Very high BP • Kidneys may be affected	**RISK** Risk for decreased cardiac output as evidenced by risk factors: increased afterload.	• Urine output above 0.5 mL/kg/hour and no chest pain.	• Follow urine output and renal function. • Watch the ECG. • Report chest pain or breathlessness.	• Achieved with adequate output and no chest pain.

*References for this care plan: [35], [37], [128], [132]. Full entries are listed in the References section of the CPMN book.